



## جوستاف روسي انترناشيونال - مصر (EPE) \* طلب علاج أول مرة / متردد

إسم المريض رباعياً : امانى عبدالرحمن عبدالسلام محمد

العنوان : الزهراء

الحي / المدينة : مصر القديمه

المحافظة : القاهرة

تاريخ تسجيل الإستمارة : 21-09-2026

رقم البطاقة : 27204150103688 رقم التليفون : 01003332890

العمر (تاريخ الميلاد) : 15-04-1972 المهنة : لاتعمل

### الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونك [14157]

التعليق على التشخيص : .....

### تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : ..... التوقيع : .....  
الاسم : د/ندى حسن الوظيفة : ..... التوقيع : .....  
الاسم : د/نرمين رمضان الوظيفة : ..... التوقيع : .....  
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.  
كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

### أعضاء اللجنة

أ.د. / ..... / د. / ..... / د. / .....

### قرار اللجنة

التشخيص النهائي : .....  
خطة العلاج : .....  
المستشفى : ..... قسم : .....  
جهة العلاج : ..... نفقات : .....  
ملاحظات : .....

ختم  
شعار الجمهورية

مدير عام  
مديرية الشؤون الصحية

رئيس  
المجلس الطبي المتخصص

|  |
|--|
|  |
|--|

|                                                                                   |                                                                                     |
|-----------------------------------------------------------------------------------|-------------------------------------------------------------------------------------|
| <b>Name:</b> امانى عبدالرحمن عبدالسلام محمد<br><b>National ID:</b> 27204150103688 |  |
|-----------------------------------------------------------------------------------|-------------------------------------------------------------------------------------|

|           |
|-----------|
| تقرير طبي |
|-----------|

|                  |
|------------------|
| السيد            |
| تحية طبية وبعد,, |

|                       |                       |
|-----------------------|-----------------------|
| إيماء للطلب المقدم من | بشأن تقرير طبي للمريض |
| بقسم:                 | رقم طبي:              |

|                                          |
|------------------------------------------|
| نتشرف بأن نوضح فيما يلي تقريراً عن حالته |
|------------------------------------------|

|                                             |                                             |
|---------------------------------------------|---------------------------------------------|
| تاريخ وساعة الدخول :<br>-- --:-- yyyy-mm-dd | تاريخ وساعة الخروج :<br>PM 01:17 05-07-2026 |
|---------------------------------------------|---------------------------------------------|

|                                        |
|----------------------------------------|
| وبعد الكشف والفحوصات والمتابعة وجد أنه |
|----------------------------------------|

|                                                                                                              |
|--------------------------------------------------------------------------------------------------------------|
| المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy |
|--------------------------------------------------------------------------------------------------------------|

|                         |                                   |
|-------------------------|-----------------------------------|
| التاريخ :<br>05-07-2026 | الوقت :<br>-- --:--               |
|                         | الطبيب المعالج :<br>د/شيماء لاشين |

جمهورية مصر العربية  
بمطابقة تحقيق الشخصية

امثلة  
عبد الرحمن عبد السلام محمد  
٨ ميدان عبدالخالق صبر-الزهره  
مصر القديمة - القاهرة  
٢٧٢.٤١٥.٠١.٣٦ ٨٨

LC5590372

٢٠٢٥/٠٤ ٢٧٢.٤١٥.٠١.٣٦٨٨  
حاصله على حقوق في صناديق  
إتشي  
مستفيدة  
محمد علي محمد عبدالواحد  
البطاقة سارية حتى ٢٠٢٦/٠٤/٨٨

١

Date Of Submission:

Date Of MDT: 04/27/2026 15:36:36

**GUSTAVE /  
ROUSSY**  
INTERNATIONAL  
EGYPT

## General GRI-E MDT form

|                     |                                                                    |                  |                                                                                                      |
|---------------------|--------------------------------------------------------------------|------------------|------------------------------------------------------------------------------------------------------|
| Patient Name :      | Amany Abdelrahman Abdels                                           | Age :            | 54 Y                                                                                                 |
| Sex :               | <input type="radio"/> Male <input checked="" type="radio"/> Female | Comorbidity :    |                                                                                                      |
| ID No. :            | 1421                                                               | Telephone :      | 01003332890                                                                                          |
| Family History :    |                                                                    | Marital Status : | <input checked="" type="radio"/> Married <input type="radio"/> Divorced <input type="radio"/> Single |
| Menopausal Status : |                                                                    | Reason of MDT :  | PD                                                                                                   |
| Menopausal Status : | MDT                                                                | MDT name::       |                                                                                                      |

## Medical Report

Diagnosis :  
Notes :

History:

patient underwent left MRM in 2018, followed by AC x4 then taxol x 12 then PORT. she started tamoxifen in 3/2018, stayed ON TAM for 5 yrs, developed bony mets + DVT in 02/2025 so shifted to AI + Ibrance, IN 09/2025 patient suffered from a visceral crisis (lower mets) so she was shifted to xeloda till now, CT 4/2026 > PD of HFL 9.4x7.1 vs 7.3 x 5.3 cm, BS > SD.

Investigations:

PS 2, pt in pain and c/o pf recurrent dizziness.  
LFTS mildly elevated but accepted, s creat 1.2-1.4

Recommendations:

for plan of management regarding bone progression

MDT DECISION:

START TAXOL WEEKLY WITH LIVER SUPPORT and Baseline CEA -CA15.3

Signature

Hematology

Pediatric

Bone marrow transplant

د. نوره مصلحي د. / نوري حسن

د. نوره مصلحي د. / نوري حسن

|                                                       |                                   |
|-------------------------------------------------------|-----------------------------------|
| Patient Name : الأستاذة / أماني عبد الرحمن عبد السلام | Request No. : 13000268            |
| Patient No. : 27204150103688                          | Request Date : 28/04/2025         |
| Path No : 1509/25                                     | Verification Date : 05/05/2025    |
| Sex : Female                                          | Financial Category : مريض - عيادة |
| Age : 53 Y                                            | Department : Out Patient          |
| Referring Doc :                                       | Registered by : Mrs.Mayada H      |

### Pathology Report

#### Clinical data

Hepatic focal lesion. A known case of breast cancer.

#### Nature of specimen

Tru cut biopsy, collected at 27/4/2025.

#### Gross examination

Three grayish white tissue cores up to 1.5 cm, totally submitted.

Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), GATA3 & Ki clone (30-9)

#### Microscopic examination

- Histological type: Metastatic adenocarcinoma.
- Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade II .
- IPT (Positive control showed appropriate reactivity):
- GATA3: Diffuse strong positive.
- ER: Moderate nuclear reaction in 80% of tumor cells (score 7/8).
- PR: Strong nuclear reaction in 20% of tumor cells (score 6/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki 67: 25 % nuclear reaction in tumor cells.

#### Diagnosis

Hepatic focal lesion, tru cut biopsy:

Metastatic adenocarcinoma grade II of primary mammary origin.

IPT:

- GATA3: Diffuse positive.
- ER: Positive (score 7/8).

Verified by :

dr.esraa magdy

Printing Date : 07/05/2025 10:36:08

Page 1 of 2

Printing By : dr.esraa magdy

مركز تشخيص الأمراض - مصر الجديدة - القاهرة - مصر  
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadima - Cairo - Egypt

|                                                        |                   |
|--------------------------------------------------------|-------------------|
| Patient Name: Amani Abd Al Rahman Abd Al Salam Mohamed | Date : 08/12/2025 |
| Exam : CT CAP+C (CT Chest Abd. Pelvis)                 | Age : 53 Year(s)  |
| Medical ID: 699/2025                                   | Pat. ID: 98734    |

**MULTISLICE POST CONTRAST CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:**

**TECHNIQUE:** Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

**FINDINGS:**

**Status post left mastectomy .**

**Compared to the last study dated 28/8/2025 ,the current study revealed:**

**(I) Chest:**

- Still noted left upper lobar anterior segmental and left superior lingular subpleural fine reticulations of post-radiation therapy sequel.
- Still noted two right lower lobar apical segmental pleural-based tiny pulmonary nodules are noted.
- Patent central air ways with well inflated both lungs.
- The rest of both lung fields are clear showing no parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- No CT evidence of hilar or mediastinal lymph node enlargement.
- Mild basal pericardial thickening is noted.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

**(II) Abdomen and pelvis:**

- The liver is enlarged in size, showing homogenous CT density with multiple bilobar marginally enhancing focal lesions, with no appreciable changes regarding the largest is seen at segment VII , measuring 7.6 x 5.6 cm compared to 7.3x5.3 cm in diameter in the previous study .
- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.

- Normal CT appearance of the spleen( with small splenule is still noted ) , pancreas, suprarenal glands, aorta and IVC.
- Horse-shoe kidneys.
- Still noted regular filling of the urinary bladder showing mild circumferential mural thickening measuring 5 mm at maximum thickness ,yet with no stones, masses or diverticulae.
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- More sclerosis of the previously noted widespread vertebral, sternal and iliac mixed lytic and sclerotic osseous lesions .
- D9 vertebral wedging

**OPINION:**

**Status post left mastectomy showing :**

- **More sclerosis of the previously noted widespread vertebral, sternal and iliac mixed lytic and sclerotic osseous lesions .**
- **Rather stationary rest of the study .**

*Dr.Reham Khedr MD*

Signature reham kheder

12/12/2025 2:43 PM

Patient's Name: AMANY ABD AL RAHMAN A.SALAM  
Date: 2026-04-12  
Referred by: Prof.Dr. CAT CAP IV  
Hospital: Goustave Roussey International Egypt

[REDACTED]  
[REDACTED] Multislice axial CT images of the chest, abdomen and pelvis obtained from the lower neck to the symphysis pubis with multiplanar reconstruction. IV contrast was used.

In reviewing the previous study report dated 28-8-2025 ,the current study revealed:

**(I) Chest:**

- **Still noted** left upper lobar lingular subpleural fine reticulations of post therapy sequel.
- Left upper lobar apico-posterior segment tiny subpleural pulmonary nodule
- The rest of both lung fields are clear showing no sizable parenchymal nodules, cavitary lesions, consolidative patches or bronchiectatic changes.
- No CT pathological evidence of hilar or mediastinal lymph node enlargement.
- No pleural or pericardial collections seen.
- No gross cardiomegaly

**(II) Abdomen and pelvis:**

- The liver is enlarged in size, showing homogenous CT density with progressive course of the previously noted multiple bilobar marginally enhancing focal lesions, with the largest is seen at segment VII , measuring about 9.4x7.1 in diameter ,compared with 7.3x5.3 cm cm in the previous study .

Patient's Name: AMANY ABD AL RAHMAN A.SALAM  
Date: 2026-04-12  
Referred by: Prof.Dr. CAT CAP IV  
Hospital: Goustave Roussey International Egypt

- Patent and homogenously enhancing portal vein.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- A splenule is seen at the splenic hilum.
- Horse-shoe kidneys.
- Regular filling of the urinary bladder showing mild circumferential mural thickening yet with no stones, masses or diverticulae.
- Unremarkable other pelvic organs.
- No retro-crural, para-aortic or pelvic lymphadenopathy.
- No ascites.
- Widespread vertebral and iliac mixed lytic and sclerotic osseous lesions are noted with still noted D6 extra-osseous intraspinal soft tissue component causing neural compromise and D9 structural collapse.

**Status follow up showing the above described changes with progressive course regarding hepatic focal lesions**

*Dr.Nourhan Ewis*

*Dr.Nourhan Ewis*

|                                               |                   |
|-----------------------------------------------|-------------------|
| Pat. Name: Amani Abd El Rahman<br>Abdel Salam | Pat. ID: 14261970 |
| Procedure Name: bone Scintigraphy             |                   |

Nuclear Medicine Unit.

### ISOTOPIC BONE SCAN REPORT

#### Clinical History

A 54-year-old female patient, with history of breast carcinoma; referred for follow up assessment.

Prior bone scan (18/12/2025, images were not provided):

Multiple metastatic osseous lesions (stationary course) involving multiple skull bones, multi-level of the spine, multiple bilateral ribs, right scapula, multiple pelvic bones (including sacrum, and sternum with the left sacral lesion is the sizable and most active lesion).

#### Technique

Bone scintigraphy done 2 hrs. Following I.V injection of 20 mCi of Tc-99m M.D.P.

#### FINDINGS

Compared to the prior study (18/12/2025), the current study reveals:

- More or less stationary course of the priory reported multiple metastatic osseous lesions (as regard tracer uptake intensity, lesion size, number and distribution) involving multiple skull bones, multi-level of the spine, multiple bilateral ribs, right scapula, multiple pelvic bones (including sacrum, and sternum with the left sacral lesion).
- No definite new lesions could be identified.
- Osteoarthritic changes are noticed over the big joints.

#### IMPRESSION

- Rather stationary course of the old metastatic osseous lesions as described, for clinical correlation and follow up.

Thanks for reference,  
Dr. Adel Khedr, MD (A.Kh)  
Dr. Mohamed Elbakhery, MSc

|                           |                  |
|---------------------------|------------------|
| Abdel Salam               |                  |
| Procedure Name: Bone Scan | Date: 18/12/2025 |
|                           |                  |

### Tc-99m MDP BONE SCAN:

#### PROCEDURE:

- Whole body skeletal survey and static spot views were obtained 3 hours post tracer injection.

#### FINDINGS:

Compared to the previous scan dated 11.9.2025 , the current study revealed

- Stationary course of the multiple, randomly distributed bone metastases involving multiple skull bones, multiple cervico-dorso-lumbar vertebrae , multiple ribs bilaterally, right scapula, multiple pelvic bones including the sacrum and sternum with the left sacral lesion is the sizable and most active lesion
- stable low grade uptake at the proximal left femur ? low grade lesion ? insignificant for follow up

#### OPINION:

- Stationary course of the bone metastases

Best Regards,  
 Prof. Dr. Maha Abd Elkareem  
 Professor of Nuclear Medicine  
 Kasr Al Ainy, Cairo University

For any inquiries regarding the findings in the report please contact us on  
[radreport@cairoscan.com.eg](mailto:radreport@cairoscan.com.eg)

Patient Name : الأستاذ / أماني عبد الرحمن عبد السلام  
Patient No. : 27204150103688  
Path No. : 1509/25  
Sex : Female  
Age : 53 Y  
Referring Doc :

Request No. : 13000268  
Request Date : 28/04/2025  
Verification Date : 05/05/2025  
Financial Category : جوشاف روسي - مبلره  
Department : Out Patient  
Registered by : Mrs.Mayada Hegazy

### Pathology Report

#### Clinical data

Hepatic focal lesion. A known case of breast cancer.

#### Nature of specimen

Tru cut biopsy, collected at 27/4/2025.

#### Gross examination

Three grayish white tissue cores up to 1.5 cm, totally submitted.

Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), GATA3 & Ki67 clone (30-9)

#### Microscopic examination

- \* Histological type: Metastatic adenocarcinoma.
- \* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2) Grade II.
- \* IPT (Positive control showed appropriate reactivity):
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- ER: Moderate nuclear reaction in 80% of tumor cells (score 7/8).
- PR: Strong nuclear reaction in 20% of tumor cells (score 6/8).
- Her2: No membrane reaction in tumor cells (score 0).
- Ki 67: 25 % nuclear reaction in tumor cells.

#### Diagnosis

Hepatic focal lesion, tru cut biopsy:

Metastatic adenocarcinoma grade II of primary mammary origin.

#### IPT:

- GATA3: Diffuse positive.
- ER: Positive (score 7/8).

Verified by :

dr.esraa magdy



Patient Name : الأستاذ / أماني عبد الرحمن عبد السلام  
Patient No. : 27204150103688  
Path No. : 1509/25  
Sex : Female  
Age : 53 Y  
Referring Doc :

Request No. : 13000268  
Request Date : 28/04/2025  
Verification Date : 05/05/2025  
Financial Category : جراحات - مفاصل  
Department : Out Patient  
Registered by : Mrs.Mayada Hegazy

### Pathology Report

- PR: Positive (score 6/8).  
- Her2: Negative (score 0).  
- Ki 67: 25 % nuclear reaction in tumor cells.  
\* Scoring system: "ER, PR"  
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.  
\* Scoring system: "Her2"  
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within < 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense < 10% of tumor cells.

Dr. Marwa Mahmoud  
Dr. Esraa Magdy

Verified by :  
dr.esraa magdy

Printing Date : 08/09/2026 11:19:10

Page 2 of 2

Printing By : All mostafa



Dr.

## Elia Anis Ishak

M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University  
Fellow, Armed Forces Institute of Pathology , USA  
Member, United States Canadian Academy of Pathology

Dr.

## Shady Elia Anis

M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University  
Fellow , Mass General Hospital , Harvard , USA  
Member , United States Canadian Academy of Pathology

Patient Name : امانى عبد الرحمن عبد السلام محمد

Analysis ID : 1093419

Age : 54 Y : 4 M : 2 D

Gender : Female

Conf. Code : 2741

Referred By Prof. Dr. : دينا كرم

Clinical Diagnosis : A case of breast cancer, presented with hepatic focal lesion

Nature of Specimen : Referred block [labelled as 1509/25]

Receiving Date : 2026-09-08

Patient ID : 797119

## IMMUNOHISTOCHEMISTRY REPORT

### Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain.  
The sections were treated against:  
- PTEN

### Results :

PTEN is RETAINED in tumor cells.

### Conclusion :

Referred block, PTEN IS RETAINED IN TUMOR CELLS.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

*Shady Elia*

*Elia anis*

*This report is electronically signed, any abrasion or modification would render it void.*



## TEST RESULT REPORT

Test performed at , Ramis Square, Cairo, Egypt.

|              |                                        |
|--------------|----------------------------------------|
| Sample ID    | Amany Abdelrahman Abdelsalam - 1093418 |
| Sample type  | FFPE tissue                            |
| Cartridge ID | 69910479                               |
| Test type    | PIK3CA-AKT1_RUO/1.2                    |
| Lot ID       | 00006991                               |
|              | TTP Version                            |
|              | 1.17                                   |
|              | Expiration date                        |
|              | 30 Apr 2027                            |

|                             |                                                 |
|-----------------------------|-------------------------------------------------|
| Instrument serial number    | SER3477                                         |
| Instrument software version | 28.0                                            |
| Console Serial Number       | C00001784                                       |
| Console software version    | 4.5.0.764                                       |
| Test Request completed      | 09 Sep 2026 (18:39)                             |
| Test started                | 09 Sep 2026 (18:39)                             |
| Test ended                  | 09 Sep 2026 (21:03)                             |
| Test status                 | Released result: Automatic, 09 Sep 2026 (21:03) |
| Operator                    | admin                                           |

Test Result For Research Use Only. Not for use in diagnostic procedures.

### Idylla™ PIK3CA-AKT1 Mutation Assay

|                                  |                                                                               |
|----------------------------------|-------------------------------------------------------------------------------|
| PIK3CA GENOTYPE                  | NO MUTATION DETECTED                                                          |
| AKT1 GENOTYPE                    | NO MUTATION DETECTED                                                          |
| Median Cq of the PIK3CA controls | 23.6                                                                          |
| Cq of the AKT1 control           | 24.3                                                                          |
| Quality Status                   | The assay controls have been properly amplified. All assay results are VALID. |